

SURGICAL DISCHARGE SUMMARY

Patient Name: Kunal M. Trivedi (Synthetic)	UHID / IP No: CCMH-AHM-40192
Age / Gender: 26 Years / Male	Consultant: Dr. Nilesh Parikh, MS, FMAS (General & Laparoscopic Surgery)
Date of Admission: 03-Oct-2026	Date of Discharge: 05-Oct-2026

Final Diagnosis

Acute Appendicitis (Operated: Laparoscopic Appendectomy)

Surgical Procedure Done

Emergency Laparoscopic Appendectomy under General Anaesthesia on 03-Oct-2026. Intraoperatively, inflamed, retrocecal appendix with fibrinous exudate noted. Appendix ligated and excised. Hemostasis achieved. Specimen sent for histopathological examination.

Postoperative Hospital Course

Patient made an uneventful postoperative recovery. Bowel sounds returned on POD-1, passed flatus, and was started on oral liquids progressing to soft diet. Surgical wounds are clean and dry, no soakage. Afebrile, vitals stable, ambulating independently.

Postoperative Discharge Medications

- Tablet Cefuroxime Axetil 500 mg - 1 tablet twice daily after food for 5 days
- Tablet Metronidazole 400 mg - 1 tablet three times daily after food for 5 days
- Tablet Paracetamol 650 mg + Tramadol 37.5 mg - 1 tablet SOS for post-op surgical site pain
- Tablet Pantoprazole 40 mg - 1 tablet once daily before breakfast for 7 days

Wound Care & Instructions

- Keep laparoscopic port-site dressings clean and dry; do not take bath until skin sutures are removed
- Avoid lifting heavy weights (> 5 kg) or strenuous abdominal straining for 3 weeks
- Light, non-spicy, high-fiber diet to avoid constipation

Follow-up Appointment

Review in Surgical OPD on 12-Oct-2026 at 10:00 AM for suture removal and review of histopathology biopsy report.

Emergency Warning Signs

Report to surgical casualty immediately if high fever (> 101 F), persistent vomiting, severe worsening abdominal pain, or purulent wound discharge occurs.